



GUIDELINE

Aboriginal Mental Health Service

Scope (Staff):	All staff
Scope (Area):	CAMHS wide

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

Contents

Acknowledgement of Country and People	2
Using the Term Aboriginal.....	2
Aim	3
Risk.....	3
Background.....	3
Definitions	3
Service Overview	5
Service Objectives	5
Principles Supporting Service Delivery.....	5
Service Delivery	6
Choice Appointment process	7
Clinical and cultural consultation liaison	7
CAMHS acute services	7
Community outreach.....	7
Recovery-based cultural activity.....	7
Workforce Development.....	7
Confidentiality	8
Team Engagement	8
Home and Community Visits	8

Acute Services	8
Documentation.....	9
Responsibilities of the Aboriginal Mental Health Worker	9
Responsibilities of the Aboriginal Mental Health Coordinator	10
Responsibilities of the Service Manager (Community) and Coordinator of Nursing (SRN7 – ICAMHS Central)	10
Infrastructure, Information and Communication Technology (ICT) and Fixed Furnishings Equipment (FFE).....	11
Quality, research and evaluation.....	11
Learning and Development	11
Key Performance Indicators.....	12
Appendix 1: Supervision Record of Attendance Register	15

Acknowledgement of Country and People

The Child and Adolescent Mental Health Service (CAMHS) acknowledges the Traditional Custodians of the Whadjuk Noongar lands and the Aboriginal people of the many Traditional Lands and language groups of Western Australia. We acknowledge the wisdom of the Aboriginal Elders, past, present, and emerging, and pay our respects to the Aboriginal Communities of today.

Using the Term Aboriginal

Within Western Australia and within this document, the term/word Aboriginal is used in preference/reference to Aboriginal and Torres Strait Islander people, in recognition that Aboriginal people are the original inhabitants of Western Australia. Aboriginal and Torres Strait Islander may be referred to in the national context and Indigenous may be referred to in the international context. No disrespect is intended to our Torres Strait Islander Colleagues and Community.

Aim

To summarise the objectives of the CAMHS Aboriginal Mental Health Service (AMHS); and describe the responsibilities of the Aboriginal Mental Health Worker (AMHW), Aboriginal Mental Health Co-ordinator (AMHC), Service Manager and Coordinator of Nursing Acute ICAMHS Central.

Risk

- A lack of clarity in the role and responsibilities of AMHW resulting in underutilisation and poor mental health outcomes for Aboriginal children, young people, and their families.
- Inconsistent work practices, roles, and responsibilities performed by AMHW, AMHC and Service Manager.
- Failure to provide adequate support to AMHW resulting in higher workplace stress and skilled worker attrition.

Background

The Mental Health Commission (MHC) funds CAMHS to deliver mental health services to Aboriginal children, young people and their families living within WA. In this document, this service is referred to as the CAMHS Aboriginal Mental Health Service (AMHS) or Waalbirnininy Wirran (healing spirits).

Definitions

Aboriginal Mental Health Co-ordinator: Provides cultural support and supervision of the AMHW workforce; revision and monitoring of policy; and provides reporting to relevant agencies such as MHC & State-wide Specialist Aboriginal Mental Health Service. Provides coordination for interagency initiatives, and for the CAMHS Aboriginal Cultural Reference Group. Conducts recruitment and budget monitoring and reporting. Provides cultural leadership to the mental health service in consultation with the CAHS Director of Aboriginal Health.

Aboriginal Mental Health Worker: Participates alongside clinicians in the planning and implementation of individual treatment plans and provides an active supportive role in the care and treatment of Aboriginal children and young people.

Child and young person: An infant, child, adolescent, or young person under the age of 18 years accessing CAMHS services.

Clinician: Term used to include psychologists, case managers, doctors, psychiatrists, social workers, nurses, occupational therapists, speech pathologists and other professionally accredited staff within mental health service that take primary or direct responsibility for the provision of health care.

Clinical capacity: AMHW works co-operatively with other members of the mental health clinical team to enhance the provision of appropriate mental health assessment, intervention, treatment and other discharge planning for Aboriginal children, young people, and their families. The AMHW does not provide direct clinical mental health intervention or treatment.

Country: As used by Aboriginal people in a cultural context and is defined as the geographic tribal area the person identifies as their ancestral and cultural homelands.

Culture: A 'set of guidelines (both explicit and implicit) which individuals inherit as members of a particular society, which tells them how to behave in it in relation to other people, to supernatural forces or gods and to the natural environment'⁴.

Cultural competency: The ability 'to see beyond the boundaries of (one's) own cultural interpretations, to be able to maintain objectivity when faced with individuals from cultures different from (one's) own and to be able to interpret and understand behaviours and intentions of people from other cultures non-judgementally and without bias'. Cultural competence is a developmental process that evolves over an extended period. Both individuals and organisations are at various levels of awareness, knowledge, and skills along the cultural competence continuum. Cultural competence requires that organisations:

- Have a defined set of values and principles, and demonstrate behaviour, attitudes, policies and structures that enable them to work effectively cross- culturally.
- Have the capacity to (1) value diversity, (2) conduct self-assessment, (3) manage the dynamics of difference, (4) acquire and institutionalise cultural knowledge, and (5) adapt to diversity and the cultural contexts of the communities they serve.
- Integrate the above in all aspects of policy making, administration, practice, service delivery and involve systematically consumers, key stakeholders, and communities.

Culturally informed practice: A holistic approach to service delivery that is inclusive of the cultural beliefs, spiritual beliefs, values, practices, and language needs of the consumer including their family, carers, and community.

Cultural security: A commitment to the principle that the construct and provision of services offered by the mental health system will not compromise the legitimate cultural rights, views, values, and expectations of Aboriginal people. It is a recognition, appreciation, and response to the impact of cultural diversity on the utilisation and provision of effective clinical care, public health and health systems administration¹.

Cultural supervision: A group session facilitated by a senior Aboriginal clinician that provides AMHW an opportunity to discuss cultural and clinical concerns in a culturally safe space. See [Appendix 1: Supervision Record of Attendance Register Template](#).

Cultural therapeutics: The use of traditional healing techniques by applying ancient cultural knowledge to address trauma and restore and sustain holistic wellbeing.

Lore: A body of traditions and knowledge on a subject or held by a particular group, typically passed from person to person by word of mouth. Lore refers to the stories, customs, beliefs, and spirituality of Aboriginal people, that was given to them from the Dreaming.

Service Manager: Responsible for the clinical and corporate governance of the service; and managing the financial, physical, and human resources of the service. Undertakes direct line management responsibility for multidisciplinary team and other staff working within the service. Represents CAMHS and liaises with senior management in other health industry agencies on broader mental health service planning and delivery.

Service Overview

The Aboriginal view of mental health is a holistic one, as embodied in the general definition of health as ‘...not just the physical wellbeing of the individual but the social, emotional, and cultural wellbeing of the whole community...’. This whole-of-life view also includes the cyclical concept of life-death-life’. Therefore, the use of the term ‘Social and Emotional Wellbeing’ reflects an increasing understanding of the need to recognise the Aboriginal holistic concept of mental health and encompasses a broader view of mental wellbeing than implied by traditional psychiatric definitions.

The CAMHS AMHS provides a whole-of-family and community engagement approach for Aboriginal children and young people in contact with CAMHS. Service delivery involves the individual’s family and community networks, including the involvement of elders and traditional healers significant to the person and their families.

This approach is inclusive of cultural beliefs, spiritual beliefs, values, practices, and the language needs of health service users. This holistic approach aims to increase cultural security within mainstream health services.

The CAMHS AMHW works across all CAMHS directorates and sites. The service provides support to children, young people, and families across the continuum of care, and collaborates with Non-Government Organisations, Aboriginal Community Controlled Organisations, Aboriginal Community Controlled Health Organisations and General Practitioner services to optimise health outcomes.

Service Objectives

The objectives of the CAMHS Aboriginal Mental Health Service is to improve mental health outcomes for Aboriginal children and young people who come into contact with CAMHS by:

- Developing, maintaining and strengthening relationships with the Aboriginal community to support access to mental health services.
- Improving access to culturally appropriate mainstream mental health services for Aboriginal children, young people and their families.
- Developing, maintaining and strengthen interagency partnerships to ensure a more holistic approach to Aboriginal mental health care.
- Supporting improvement of Aboriginal community awareness of mental health issues affecting Aboriginal children and young people.
- Improving the cultural knowledge and skills of mainstream CAMHS workforce.

Principles Supporting Service Delivery

- The AMHW is to be engaged and utilised in the Aboriginal child and young person’s journey through CAMHS from referral, assessment/formulation, diagnosis, management planning and review, to discharge planning, transfer of care and post discharge into community.

- A key function of the AMHW is to build trust and rapport within community recognising the centrality of whole of family engagement and the respect of Aboriginal spirit and lore as an integral part of mental health service delivery.
- The AMHW role is to actively present within their local community liaising with Aboriginal families, government and non-government agencies and services identifying and supporting Aboriginal children and young person's mental health needs.
- The AMHW ICAMHS Central will work within CAMHS acute services and be an active participant in the health care journey of Aboriginal children, young people, and their families.
- The multidisciplinary team and CAMHS Leadership support the AMHW in integrating the cultural therapeutic needs of Aboriginal children, young people, and families that may include incorporating traditional healing practices, Elders and utilising on country recovery-based cultural activities (CAMHS on country).
- To support of the cultural safety needs of consumers, carers and family as best as practicable, the AMHS allocates the appropriate gender and cultural identity/mob of the AMHW.
- The AMHW, in conjunction with the multidisciplinary team, advocate for the spiritual needs of Aboriginal children, young people, and their families to be considered in the mental health care plan where appropriate.
- The AMHW participates as an active member of the multidisciplinary team by attending team meetings and case reviews.
- The AMHW will be embedded in existing CAMHS community and acute services and will be subject to the same governance procedures as currently exist within the service.
- The AMHW works co-operatively with other members of the mental health clinical team to enhance the provision of appropriate mental health assessment, crisis intervention and treatment for Aboriginal children, young people, and their families.
- The AMHW provides advice to the mental health team on Aboriginal cultural and local family issues.
- The AMHW supports Aboriginal children, young people, and their families in their concerns with mental health services and educates Aboriginal people in the benefits of involving themselves in their ongoing treatment.
- The AMHS contributes to the development and delivery of appropriate recovery focused mental health programs.

Service Delivery

Services delivered by CAMHS AMHS Waalbirnininy Wirran, will include the following and remain consistent with and build upon the existing CAMHS service delivery model.

Choice Appointment process

The AMHW may provide information regarding the process prior to the Choice Appointment and where necessary, may accompany families to the appointments. Where other services are deemed to be more clinically appropriate, AMHW's can ensure linkages with the identified services so as to avoid any gaps and provide a seamless transition into an appropriate service. The AMHW can also liaise with PCH CAMHS services and advocate for Aboriginal children, young people, and their families to access out-of-hours services, where necessary.

Clinical and cultural consultation liaison

The AMHW will offer consultation to other clinicians within CAMHS and other agencies in the community (e.g. GP's) where Aboriginal children, young people, and their families present with mental health concerns. The AMHW workers will provide cultural input to CAMHS clinicians who will continue to remain the nominated case managers.

CAMHS acute services

The AMHW ICAMHS Central will work within the CAMHS acute services and will be guided by models of care and clinical governance specific to programs for Ward 5A, Gender Diversity Service, Eating Disorders Program, CAMHS Crisis Connect. PCH Psychiatric In-Reach Team. The AMHW will provide consultation, liaison, support and advocacy to Aboriginal children and young people accessing acute services.

Referrals for these services will be emailed to AMHWICAMHScentral@wa.gov.au. The email will be monitored by the AMHC who will liaise with the AMHW and Coordinator of Nursing to allocate and monitor workload.

Community outreach

The CAMHS AMHW will provide outreach services to the local Aboriginal community including supporting the facilitation of assessment and treatment at suitable locations outside of CAMHS facilities, where appropriate.

Recovery-based cultural activity

The staff will access existing links to Aboriginal mental health literacy and education as well as develop CAMHS specific mental health literacy, where appropriate for access by other agencies working with Aboriginal children, young people, and their families. Community education within the Aboriginal community will aim to de-stigmatise mental illness and facilitate early recognition and treatment of mental disorders.

Workforce Development

The Aboriginal workforce will be developed and upskilled to participate in specialist CAMHS assessments and treatments. This will be achieved by providing the staff with existing CAMHS orientation and training. Opportunities will be provided to the Aboriginal workforce to access appropriate career development pathways. Upskilling and enhancing the cultural competence of non-Aboriginal workers by providing cultural information, support, supervision, and education will be achieved by accessing and participating in cultural competency training.

Confidentiality

- AMHWs are subject to the same confidentiality policies and guidelines as other CAMHS staff.
- Members of Aboriginal communities are expected to abide by confidentiality relating to cultural business (such as lore-time or gender specific business). This may result in a conflict of interest for an AMHW whereby, in the course of their duties, they are privileged with/or granted access to culturally significant information that is not appropriate to be recorded in any format. Fidelity to the confidentiality can become a source of tension for AMHW's who want to abide by the Aboriginal community expectations and the mental health service clinical requirements.
- When an AMHW has specific cultural information that is not for documenting the AMHW is required to:
 - Discuss the parameters of this confidential information but not the cultural information itself with the Clinical Lead to resolve any medicolegal and duty of care responsibilities; and
 - Work with the AMHC to identify the most appropriate means of recording this information whilst also maintaining cultural confidentiality.

Team Engagement

Depending on the context (e.g. inpatient versus outpatient setting) and team requirements, the AMHW may participate in the following:

- Daily Multidisciplinary Team (MDT) meetings
- Carer support groups
- Business meetings
- Individual case reviews and case conferences

AMHW's will ensure effective communication between CAMHS and external health services when Aboriginal children and young people are transitioning care..

Home and Community Visits

- The AMHW conducting home and community (e.g. school) visits must do so in line with the [CAMHS Home and Community Visits Procedure](#).
- The AMHW is encouraged to be in community as much as practicable which is likely to include making home visits.
- Service Managers are to ensure that AMHW's are supported and feel safe in conducting home and community visits.

Acute Services

- The AMHW ICAMHS Central will work across all services within CAMHS acute services.

- All referrals to the AMHW will be via the AMHWICAMHScentral@wa.gov.au email address.
- The AMHW will be responsible for liaising with referrers to organise reviews in a timely manner, priority for reviews will be based on clinical need at the time
- The AMHW will access monthly Cultural Supervision in support of their practices with the AMHC.

Documentation

- All care provided by the AMHW will be documented in the consumer's health record (including PSOLIS/webPSOLIS), as per the CAMHS Medical Records Management Procedure.
- The AMHW must understand their responsibilities regarding entering of information into the PSOLIS.
- The AMHW will be responsible for ensuring they comply with all documentation process requirement subject to their role.
- The AMHW ICAMHS Central will be required to enter data into the DMR application.

Responsibilities of the Aboriginal Mental Health Worker

The AMHW will:

- Provide a first or early point of contact for Aboriginal children, young people, and their families who are referred to the service.
- Make initial contact upon referral and explain the service to the families where feasible and appropriate.
- Support children, young people and their families journeying through CAMHS acute services.
- Where needed, accompany the families, to their appointments at CAMHS locations or other locations suitable for the provision of CAMHS services, including domiciliary visits, in accordance with the CAHS Transport Assistance of Patient/Client Policy.
- Strive to establish close professional relations with the local Aboriginal community and Elders so as to ensure early identification of mental health issues in the children and young people of the community and strive to facilitate and support early assessment and treatment with an appropriate service.
- Enter consumer data into the Digital Medical Records system (DMR) and Psychiatric Services Online Information System (PSOLIS) as required.
- Aim to provide a cultural context to the presentation of the child, young people, and family at CAMHS and to address barriers.
- Strive to maintain effective, collaborative partnerships with local mental health services, Emergency Department health services, as well as external service

providers including General Practitioners, Alcohol and Other Drug services, Social and Emotional Well-being services, Aboriginal Community Controlled Organisations and Aboriginal Community Controlled Health Organisations. This will ensure that children and young people can receive care from whichever organisation is best placed to provide it, recognising that wherever possible, support should be provided within their own community.

Responsibilities of the Aboriginal Mental Health Coordinator

The AMHC will:

- Provide leadership, planning and development of culturally informed and culturally safe mental health services.
- Establish effective cultural supervision practices for the AMHW and record attendance at cultural supervision sessions. See [Appendix 1: Supervision Record of Attendance Register Template](#).
- Provide cultural input and apply a cultural lens over the development and review of policy, operating procedures and service planning.
- Develop, support and maintain supportive working relationships with key stakeholder agencies.
- Work in partnership with the Service Manager, to ensure that the AMHW has access to learning, development and training opportunities relevant to support their work activities.
- Coordinate recruitment and support the onboarding of the AMHW in collaboration with the Service Manager.
- Support and maintain effective communication between the AMHW, clinicians, Service Managers and other support staff.
- Support the conflict resolution process for the AMHW.
- Support the AMHW with referrals of Aboriginal consumers requiring acute services.

Responsibilities of the Service Manager (Community) and Coordinator of Nursing (SRN7 – ICAMHS Central)

The Service Manager (Community) and Coordinator of Nursing Acute (ICAMHS Central) is responsible for:

- Ensuring that teams actively engage, support and utilise the AMHW in all aspects of the Aboriginal child, young person, and family's care journey.
- Fostering, supporting and recognising that the AMHW presence within community is a critical part of their role, that brokers cultural safety for CAMHS to ensure that the service is culturally secure for Aboriginal people.
- Supporting the AMHW with their ongoing learnings of clinical practices within the CAMHS settings to support their consultation, liaison and advocacy role.

- Recognising that AMHW's are both workers and members of their community and that some cultural confidentiality and conflict of interest issues may arise due to Aboriginal kinship and cultural protocols (e.g. lore-time or gender business).
- Human Resource (HR) matters including leave, mandatory training, and performance development requirements specific to their service.
- Working closely with the AMHC to access training and support for AMHW's and engage in the recruitment process.
- Allocating tasks within the scope of the role.
- Oversight of Workplace Health, Safety and Wellbeing (WHSW).
- Supporting the facilitation of ongoing internal mental health consultation and liaison with senior clinicians to develop AMHW mental health competency.

Infrastructure, Information and Communication Technology (ICT) and Fixed Furnishings Equipment (FFE)

- All AMHW's have access to infrastructure, ICT and FFE as required, to be determined at a local level.
- Dedicated storage space for resources is to be arranged by the Service Manager.

Quality, research and evaluation

- The AMHW participates in service evaluation, quality improvement and research activities as determined via discussion with the line manager and the AMHC.

Learning and Development

- CAMHS is dedicated to the provision of professional development and training for the AMHW.
- Provision of ongoing training, education and professional development for AMHW's facilitates growth of the CAMHS Aboriginal workforce and delivery of culturally safe and responsive care.
- The AMHW participates in mandatory training required by all CAHS employees as described in the CAHS Safety Skills Training (health.wa.gov.au), and role specific training as outlined in the table below.

Table 1: Role specific learning and development requirements

Type	Frequency	Learning Outcomes
CAMHS Orientation	Once only	Awareness of CAMHS structures and processes, and models of care, policy and NSQHS Standards.
Sexual Safety on-line package	Once only	Understanding and application of the CAMHS Sexual Safety Guideline.
Clinical Skills Suite	Once only	Awareness of clinician's tools of assessment and increased capacity to understand and participate in MDT meetings, initial reviews and ongoing reviews.
Trauma informed care	Once only	Recognise and respond appropriately to people experiencing difficulty interacting with the health environment.
Mental Health Act 2014 training (OCP)	Once only	Understand the clinical tenets of the Mental Health Act 2014.
Therapeutic Crisis Intervention (TCI) (For AMHW staffing Ward 5A, EDS, Pathways and Touchstone)	Once only with annual refresher	Prevent or de-escalate crisis. Includes active listening, reflective and empathic responses and identifying and responding to feelings.
PSOLIS	Initial session with refresher training as required	Awareness of the resources, procedures, features and data entry process for the PSOLIS/webPSOLIS application.
External Training		
Gatekeeper suicide prevention training	Once only	Increased knowledge and understanding of suicidal behaviour and increased capacity to respond effectively to those in need.

Key Performance Indicators

Key Performance Indicator	Reporting cadence
Community visits conducted	6 monthly
Completion of learning and development requirements	annually
Aboriginal consumer referral	6 monthly
YES and CES scores	6 monthly

References
1. Australian Health Ministers' Advisory Council. Cultural Respect Framework 2016 - 2026 for Aboriginal and Torres Strait Islander Health . [Accessed: 02 December 2024]
2. Closing the Gap. Closing the Gap targets Analysis and Progress and Key Drivers of Change [Internet] Canberra ACT: 2019 [Accessed: 02 December 2024]
3. Ganapathi, Dr Nalina. 2019. The Need for Intercultural Communication Skills in the Multicultural Settings . Journal of General Management Research (Journal of General Management Research) 39-47
4. Government of Western Australia. Implementation Guide for the WA Aboriginal Health and Wellbeing Framework 2015-2030 . Perth (WA): Department of Health; 2017 [Accessed: 02 December 2024]
External Legislation, Standards and Policy
Chief Psychiatrist's Standards for Clinical Care – Aboriginal Practice
Mental Health Act (WA) 2014
MP0071/17 Aboriginal Health and Wellbeing Policy
MP 0097/18 Aboriginal Workforce Policy
NSQHS Standards User Guide for Aboriginal and Torres Strait Islander Health
Related CAHS internal policies, procedures and guidelines
CAHS Access and Use of Medical Records CAHS Safely Working Alone Policy CAHS Home and Community Visits Policy CAMHS Home and Community Visits Procedure CAMHS Medical Records Management Procedure CAMHS Access Policy CAMHS Home and Community Visits Procedure CAMHS Policy Manual
Useful resources (including related forms)
Aboriginal Mental Health Worker (ICAMHS Central) Work Instruction

This document can be made available in alternative formats on request.

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 Healthy kids, healthy communities CompassionExcellenceCollaborationAccountabilityEquityRespect Neonatology Community Health Mental Health Perth Children's Hospital			

Appendix 1: Supervision Record of Attendance Register

Note: Template suggested for any group or cultural supervision Session.

Supervisor Name	
Supervisor Position	
Date	

Supervisee Name	Supervisee Position

Key Area/s Discussed	Agreed Actions	Timeframes

Agenda Items for Next Session		